

Title: Grip Strength Outcomes in ADHERE and ADHERE+	
Project Owner: João Luís	Date: January 13, 2026
Agency Contact(s): N/A	

Purpose: This data on file packet is the reference for the following information: Data in this packet will provide information about grip strength outcomes observed in the ADHERE and ADHERE+ (IA2, cutoff date: 16 February 2024) clinical trials. These data provide information on: - time to first improvement by functional measure (grip strength, aINCAT, and I-RODS) - percentage of patients with evidence of clinical improvement (ECI) achieving improvement in grip strength only, improvement in grip strength + I-RODS, improvement in grip strength + aINCAT, and improvement in all three measures - magnitude and cumulative frequency of grip strength improvement from Stage A baseline through to ADHERE+ Week 36. These will be used to leverage grip strength data from ADHERE and ADHERE+ to support efgartigimod rHuPH20 SC as an effective therapy in improving and maintaining grip strength for patients with chronic inflammatory demyelinating polyradiculoneuropathy (CIDP), alongside established disability scales.
Use(s): This data may be used in the following example medical materials, including but not limited to: - MSL field materials - Medical Information documents - Frequently asked questions (FAQs) - Field readiness guide - Advisory boards - Symposia - HEOR materials These data may be used in the following example commercial materials, including but not limited to: - Pre-approval information exchange (PIE) and payer value proposition (PVP) decks - Commercial interactive visual aid (CIVA) - Frequently asked questions (FAQs) - HCP marketing materials - Patient marketing materials - Speaker decks - Printed leave behinds

Content: <u>Speed of Response</u> <u>Time to 8 kPa grip strength improvement in either hand</u> - A total of 322 patients received efgartigimod rHuPH20 SC in Stage A. There were 240 patients (75.7%) with an 8 kPa improvement in mean grip strength from Stage A baseline in either hand to any time during Stage A, Stage B, or ADHERE+. 25% of patients achieved improvement within 1.4 weeks (95% CI, 1.3-2.1) 50% of patients achieved improvement by 4.1 weeks (95% CI, 3.1-5.1) 75% of patients achieved improvement by 13.3 weeks (95% CI: 9.9-not calculable)
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Table RQ2025023.4A: MEAN GRIP STRENGTH: Time to 8 kPa Grip Strength Improvement - Kaplan-Meier

ANALYSIS SET: Stage A Safety

	EFG PH20 SC (N=322)
25th PERCENTILE (95% CI) (WEEKS)	1.4 (1.3; 2.1)
50th PERCENTILE (95% CI) (WEEKS)	4.1 (3.1; 5.1)
75th PERCENTILE (95% CI) (WEEKS)	13.3 (9.9; NC)
NUMBER ASSESSED	317
NUMBER OF EVENTS [1]	240 (75.7%)
NUMBER CENSORED	77 (24.3%)
Discontinued from Treatment in Stage A	46 (14.3%)
Discontinued from Treatment in Stage B	3 (0.9%)
Discontinued from Treatment in ADHERE+	2 (0.6%)
Completed EFG PH20 SC Treatment per Protocol [2]	26 (8.1%)

[1] Number of participants with an 8 kPa improvement from Stage A baseline in either hand at any time during Stage A, Stage B, or ADHERE+.

[2] Includes patients who were randomized to Placebo in Stage B, who participated in the sub-study, or completed treatment in ADHERE+.

Note: The denominator for the percentage calculation is the total number of participants in the Stage A Safety Set.

Participants without an improvement before discontinuing from ADHERE+ or from Stage A or Stage B for any other reason were seen as never reaching an improvement and censored at the date of the last assessment of the last participant.

Participants who discontinued because all events for the primary analysis were achieved, were censored at the date of their last assessment.

Participants without an improvement before data cutoff date, completion of treatment, randomization to Placebo, or participation in the sub-study were censored at the date of their last assessment under EFG PH20 SC.

Program: /clinical/argx-113/cidp/argx-113-0000/biostat/staging/rq2025023/programs/ttinitconfecikm.sas, Version: 1.2

Time to 1 point aINCAT improvement

- A total of 322 patients received efgartigimod rHuPH20 SC in Stage A. There were 212 patients (67.1%) with a 1 point aINCAT improvement from Stage A baseline to any time during Stage A, Stage B, or ADHERE+.
 - 25% of patients achieved improvement within 2.1 weeks (95% CI, 2.1-3.1)
 - 50% of patients achieved improvement by 8.3 weeks (95% CI, 6.6-10.1)
 - The time by which 75% of patients achieved improvement could not be determined within the observed study period

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Table RQ2025023.4C: Time to 1 Point Adjusted INCAT Improvement - Kaplan-Meier

ANALYSIS SET: Stage A Safety

	EFG PH20 SC (N=322)
25th PERCENTILE (95% CI) (WEEKS)	2.1 (2.1; 3.1)
50th PERCENTILE (95% CI) (WEEKS)	8.3 (6.6; 10.1)
75th PERCENTILE (95% CI) (WEEKS)	NC (42.1; NC)
NUMBER ASSESSED	316
NUMBER OF EVENTS [1]	212 (67.1%)
NUMBER CENSORED	104 (32.9%)
Discontinued from Treatment in Stage A	51 (15.8%)
Discontinued from Treatment in Stage B	1 (0.3%)
Discontinued from Treatment in ADHERE+	4 (1.2%)
Completed EFG PH20 SC Treatment per Protocol [2]	48 (14.9%)

[1] Number of participants with a 1 point adjusted INCAT improvement from Stage A baseline.

[2] Includes patients who were randomized to Placebo in Stage B, who participated in the sub-study, or completed treatment in ADHERE+.

Note: The denominator for the percentage calculation is the total number of participants in the Stage A Safety Set.

Note: Participants without an improvement before discontinuing from ADHERE+, from Stage A, or Stage B were seen as never reaching an improvement and censored at the date of the last assessment of the last participant.

Participants who discontinued because all events for the primary analysis were achieved, were censored at the date of their last assessment.

Participants without an improvement before completion of treatment, randomization to Placebo, or participation in the sub-study were censored at the date of their last assessment under EFG PH20 SC.

Program: /clinical/argx-113/cidp/argx-113-0000/biostat/staging/rq2025023/programs/ttinitconfecikm.sas, Version: 1.5

Time to 4 point I-RODS improvement

- A total of 322 patients received efgartigimod rHuPH20 SC in Stage A. There were 220 patients (68.8%) with a 4 point I-RODS improvement from Stage A baseline to any time during Stage A, Stage B, or ADHERE+.
 - 25% of patients achieved improvement within 2.1 weeks (95% CI, 2.1-2.3)
 - 50% of patients achieved improvement by 6.4 weeks (95% CI, 4.4-8.1)
 - The time by which 75% of patients achieved improvement could not be determined within the observed study period

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Table RQ2025023.4D: Time to 4 Points I-RODS Improvement - Kaplan-Meier

ANALYSIS SET: Stage A Safety

EFG PH20 SC
(N=322)

25th PERCENTILE (95% CI) (WEEKS)	2.1 (2.1; 2.3)
50th PERCENTILE (95% CI) (WEEKS)	6.4 (4.4; 8.1)
75th PERCENTILE (95% CI) (WEEKS)	NC (33.0; NC)
NUMBER ASSESSED	320
NUMBER OF EVENTS [1]	220 (68.8%)
NUMBER CENSORED	100 (31.3%)
Discontinued from Treatment in Stage A	55 (17.1%)
Discontinued from Treatment in Stage B	3 (0.9%)
Discontinued from Treatment in ADHERE+	2 (0.6%)
Completed EFG PH20 SC Treatment per Protocol [2]	40 (12.4%)

[1] Number of participants with a 4 points I-RODS improvement from Stage A baseline.

[2] Includes patients who were randomized to Placebo in Stage B, who participated in the sub-study, or completed treatment in ADHERE+.

Note: The denominator for the percentage calculation is the total number of participants in the Stage A Safety Set.

Note: Participants without an improvement before discontinuing from ADHERE+, from Stage A, or Stage B were seen as never reaching an improvement and censored at the date of the last assessment of the last participant.

Participants who discontinued because all events for the primary analysis were achieved, were censored at the date of their last assessment.

Participants without an improvement before completion of treatment, randomization to Placebo, or participation in the sub-study were censored at the date of their last assessment under EFG PH20 SC

Program: /clinical/argx-113/cidp/argx-113-0000/biostat/staging/rq2025023/programs/ttinitconfecikm.sas, Version: 1.5

Grip Strength Improvement in ECI Responders

Confirmed ECI responders with improvements in mean grip strength - Stage A baseline to Stage A last assessment

ECI was defined as an ≥ 8 kPa improvement in either hand for mean grip strength, a ≥ 1 -point improvement for aINCAT, or a ≥ 4 -point improvement for I-RODS. There were 214 patients with ECI in Stage A.

- Percentage of patients achieving an ≥ 8 kPa improvement in either hand for mean grip strength: 80.4% (172/214)
- Percentage of patients achieving an ≥ 8 kPa improvement in either hand for mean grip strength and a ≥ 1 -point improvement for aINCAT: 58.4% (125/214)
- Percentage of patients achieving an ≥ 8 kPa improvement in either hand for mean grip strength and a ≥ 4 -point improvement for I-RODS: 58.9% (126/214)
- Percentage of patients achieving an ≥ 8 kPa improvement in either hand for mean grip strength, a ≥ 1 -point improvement for aINCAT, and a ≥ 4 -point improvement for I-RODS: 46.7% (100/214)

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TABLE RQ2025029.12.1: ADJUSTED INCAT, MEAN GRIP STRENGTH, AND I-RODS IMPROVEMENT FROM STAGE A BASELINE BY ECI RESPONSE - STAGE A LAST ASSESSMENT

ANALYSIS SET: SAFETY

	ECI Response (N = 214)	No ECI Response (N = 108)	TOTAL (N = 322)
8 kPa MGS	172 (80.4)	38 (35.2)	210 (65.2)
1-point aINCAT	152 (71.0)	19 (17.6)	171 (53.1)
4 points I-RODS	157 (73.4)	30 (27.8)	187 (58.1)
1-point aINCAT & 8 kPa MGS	125 (58.4)	11 (10.2)	136 (42.2)
4 points I-RODS & 1-point aINCAT	116 (54.2)	12 (11.1)	128 (39.8)
4 points I-RODS & 8 kPa MGS	126 (58.9)	19 (17.6)	145 (45.0)
4 points I-RODS & 1-point aINCAT & 8 kPa MGS	100 (46.7)	9 (8.3)	109 (33.9)

A Mean Grip Strength (MGS) improvement in either hand is counted as improvement.
The adjusted INCAT score is used to determine a 1-point aINCAT improvement.
I-RODS Centile Metric Score is used to determine a 4 points I-RODS improvement.

Magnitude of Response

Mean grip strength improvement in dominant hand - descriptive statistics of actual values and changes from Stage A baseline to ADHERE+ Week 36

As shown in Table RQ2025023.1A below:

- **Stage A baseline:** The mean (S.E.) grip strength (dominant hand) was 37.6 kPa (2.22) in the efgartigimod rHuPH20 SC group (n=111) and 40.7 kPa (2.26) in the placebo group (n=110)
- **Stage B baseline:** The mean (S.E.) grip strength increased to 54.9 kPa (2.24) for the efgartigimod rHuPH20 SC group (n=111) and 58.0 kPa (2.39) for the placebo group (n=110). The mean (S.E.) change from Stage A baseline at this point was 17.4 kPa (1.55) for efgartigimod rHuPH20 SC and 17.3 kPa (1.65) for placebo
- **Stage B last assessment:** The mean (S.E.) grip strength was 57.0 kPa (2.55) for the efgartigimod rHuPH20 SC group (n=111) and 50.3 kPa (2.69) for the placebo group (n=109). The mean (S.E.) change from Stage A baseline was 19.4 kPa (2.08) for efgartigimod rHuPH20 SC and 9.2 kPa (1.91) for placebo

In ADHERE+, all participants received efgartigimod rHuPH20 SC:

- **ADHERE+ Week 12:** The mean (S.E.) grip strength was 58.9 kPa (2.67) for those continuing efgartigimod rHuPH20 SC (n=94) and 63.4 kPa (2.67) for those who had received placebo during Stage B (n=92). The mean (S.E.) change from Stage A baseline was 22.4 kPa (2.38) for the efgartigimod rHuPH20 SC group and 22.2 kPa (2.42) for the prior placebo group
- **ADHERE+ Week 24:** The mean (S.E.) grip strength was 63.5 kPa (3.03) for those continuing efgartigimod rHuPH20 SC (n=86) and 66.3 kPa (2.91) for those who had received placebo during Stage B (n=92). The mean (S.E.) change from Stage A baseline was 25.0 kPa (2.56) for the efgartigimod rHuPH20 SC group and 25.8 kPa (2.69) for the prior placebo group
- **ADHERE+ Week 36:** The mean (S.E.) grip strength was 64.3 kPa (3.11) for those continuing efgartigimod rHuPH20 SC (n=84) and 65.6 kPa (2.93) for those who had received placebo during Stage B (n=87). The mean (S.E.) change from Stage A baseline was 25.4 kPa (2.60) for the efgartigimod rHuPH20 SC group and 24.7 kPa (2.73) for the prior placebo group

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Table RQ2025023.1A: MEAN GRIP STRENGTH (dominant hand, KPA): DESCRIPTIVE STATISTICS OF ACTUAL VALUES AND CHANGES FROM STAGE A BASELINE

ANALYSIS SET: 1802 MITT

	ACTUAL VALUES			CHANGE FROM STAGE A BASELINE		
	EFG PH20 SC (N=111)	PBO PH20 SC (N=110)	ALL PARTICIPANTS (N=221)	EFG PH20 SC (N=111)	PBO PH20 SC (N=110)	ALL PARTICIPANTS (N=221)
STAGE A BASELINE						
n	111	110	221			
Mean (S.E.)	37.6 (2.22)	40.7 (2.26)	39.1 (1.58)			
S.D.	23.33	23.72	23.53			
Median	38.0	40.5	39.0			
Q1; Q3	(19.0; 53.0)	(26.0; 53.0)	(21.0; 53.0)			
Min; Max	(0; 97)	(0; 107)	(0; 107)			
CI (95%)	(33.17; 41.96)	(36.24; 45.20)	(36.02; 42.26)			
STAGE B BASELINE						
n	111	110	221	111	110	221
Mean (S.E.)	54.9 (2.24)	58.0 (2.39)	56.5 (1.64)	17.4 (1.55)	17.3 (1.65)	17.3 (1.13)
S.D.	23.69	23.09	24.36	16.31	17.34	16.80
Median	56.0	54.5	55.0	14.0	13.0	13.0
Q1; Q3	(42.0; 70.0)	(45.0; 71.0)	(43.0; 70.0)	(5.0; 23.0)	(8.0; 23.0)	(6.0; 23.0)
Min; Max	(0; 105)	(0; 125)	(0; 125)	(-11; 72)	(-30; 79)	(-30; 79)
CI (95%)	(50.47; 59.36)	(53.27; 62.75)	(53.23; 59.69)	(14.28; 20.42)	(14.01; 20.57)	(15.09; 19.55)
STAGE B LAST ASSESSMENT						
n	111	109	220	111	109	220
Mean (S.E.)	57.0 (2.55)	50.3 (2.69)	53.7 (1.86)	19.4 (2.08)	9.2 (1.91)	14.4 (1.45)
S.D.	26.91	26.09	27.62	21.91	19.90	21.54
Median	55.0	49.0	51.0	14.0	6.0	10.0
Q1; Q3	(38.0; 78.0)	(32.0; 70.0)	(35.0; 72.5)	(4.0; 33.0)	(-1.0; 20.0)	(0.0; 25.0)
Min; Max	(2; 115)	(0; 124)	(0; 124)	(-19; 95)	(-53; 67)	(-53; 95)
CI (95%)	(51.92; 62.04)	(45.00; 55.65)	(50.01; 57.35)	(15.29; 23.54)	(5.44; 13.03)	(11.51; 17.24)

Program: /clinical/argx-113/cidp/argx-113-0000/biostat/staging/rq2025023/programs/t_eff_by.sas, Version: 0.2

ARGX-113-1802/1902 - POST-HOC ANALYSIS

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Table RQ2025023.1A: MEAN GRIP STRENGTH (dominant hand, KPA): DESCRIPTIVE STATISTICS OF ACTUAL VALUES AND CHANGES FROM STAGE A BASELINE

ANALYSIS SET: 1802 MITT

	ACTUAL VALUES			CHANGE FROM STAGE A BASELINE		
	EFG PH20 SC (N=111)	PBO PH20 SC (N=110)	ALL PARTICIPANTS (N=221)	EFG PH20 SC (N=111)	PBO PH20 SC (N=110)	ALL PARTICIPANTS (N=221)
ARGX-113-1902 WEEK 12						
n	94	92	186	94	92	186
Mean (S.E.)	58.9 (2.67)	63.4 (2.67)	61.1 (1.89)	22.4 (2.38)	22.2 (2.42)	22.3 (1.69)
S.D.	25.72	25.63	25.81	23.04	23.17	23.04
Median	56.5	60.5	59.0	18.5	16.0	16.0
Q1; Q3	(41.0; 75.0)	(44.5; 84.0)	(43.0; 77.0)	(6.0; 42.0)	(6.5; 30.5)	(6.0; 35.0)
Min; Max	(0; 128)	(3; 133)	(0; 133)	(-19; 105)	(-12; 94)	(-19; 105)
CI (95%)	(53.58; 64.20)	(58.09; 68.71)	(57.39; 64.86)	(17.73; 27.16)	(17.42; 27.02)	(19.00; 25.67)
ARGX-113-1902 WEEK 24						
n	86	92	178	86	92	178
Mean (S.E.)	63.5 (3.03)	66.3 (2.91)	64.9 (2.09)	25.0 (2.56)	25.8 (2.69)	25.4 (1.86)
S.D.	26.03	27.31	27.93	23.73	23.84	24.77
Median	61.5	61.0	61.0	19.5	19.5	19.5
Q1; Q3	(46.0; 83.0)	(48.0; 89.0)	(46.0; 83.0)	(8.0; 38.0)	(8.5; 35.0)	(8.0; 36.0)
Min; Max	(0; 127)	(3; 155)	(0; 155)	(-15; 104)	(-17; 116)	(-17; 116)
CI (95%)	(57.50; 69.53)	(60.50; 72.06)	(60.81; 69.08)	(19.95; 30.12)	(20.40; 31.10)	(21.74; 29.07)
ARGX-113-1902 WEEK 36						
n	84	87	171	84	87	171
Mean (S.E.)	64.3 (3.11)	65.6 (2.93)	64.9 (2.13)	25.4 (2.60)	24.7 (2.73)	25.0 (1.88)
S.D.	26.16	27.36	27.80	23.83	23.47	24.61
Median	66.5	61.0	63.0	18.5	18.0	18.0
Q1; Q3	(44.0; 82.0)	(48.0; 84.0)	(47.0; 84.0)	(9.5; 40.5)	(6.0; 38.0)	(8.0; 39.0)
Min; Max	(0; 128)	(0; 153)	(0; 153)	(-19; 105)	(-17; 114)	(-19; 114)
CI (95%)	(58.12; 70.47)	(59.76; 71.39)	(60.75; 69.14)	(20.23; 30.58)	(19.27; 30.13)	(21.33; 28.76)

Program: /clinical/argx-113/cidp/argx-113-0000/biostat/staging/rq2025023/programs/t_eff_by.sas, Version: 0.2

Example data presentation below:

INITIAL TREATMENT PERIOD (OPEN LABEL, STAGE A)

RANDOMIZED WITHDRAWAL PERIOD (PLACEBO CONTROLLED, STAGE B)

OPEN LABEL EXTENSION (ADHERE+)

VVVGART Hytrulo[®]
(efgartigimod alfa and hyaluronidase-qvfc)

Patients with evidence of improvement continued to improve in grip strength through Week 36 of ADHERE+

ADHERE (≤60 weeks)

ADHERE+

Mean change from ADHERE Stage A baseline in grip strength (dominant hand), kPa

Stage A Baseline

Stage B Baseline

Stage B Last Assessment

Week 12

Week 24

Week 36

n= 221

110

111

109

92

92

87

111

111

94

86

84

— Placebo SC

-- Received placebo in Stage B

— VVVGART Hytrulo

Study limitations: This was a post-hoc analysis based on a prespecified descriptive supplemental analysis not controlled for multiplicity and not powered and is based on a small sample set; therefore, data should be interpreted with caution and conclusions cannot be drawn.*

PH20, recombinant human hyaluronidase PH20; SC, subcutaneous; SE, standard error.
An increase of ≥8 kPa in grip strength¹ is considered a minimal clinically important difference.²
*This post-hoc analysis included patients with evidence of improvement from the initial treatment period (open-label, Stage A) who had a visit in the ADHERE+ open-label extension trial at Week 36. 110 patients received placebo during the randomized withdrawal period (placebo-controlled, Stage B). ADHERE+ data cut-off: February 16, 2024.
1. Vanhoutte EK. Eur J Neurol. 2013;20(5):748-55. 2. Van den Bergh PYK, et al. Eur J Neurol. 2021;28(11):3556-83.

Reference:

- ARGX-113-1802/1902 Post-hoc Analysis- Table RQ2025023.4A: Mean Grip Strength: Time to 8 kPa Grip Strength Improvement in Either Hand - Kaplan-Meier
- ARGX-113-1802/1902 Post-hoc Analysis - Table RQ2025023.4C: Time to 1 Point Adjusted INCAT Improvement - Kaplan-Meier
- ARGX-113-1802/1902 Post-hoc Analysis - Table RQ2025023.4D: Time to 4 Points I-RODS Improvement - Kaplan-Meier
- ARGX-113-1802/1902 Post-hoc Analysis - Table RQ2025029.12.1: Adjusted INCAT, Mean Grip Strength, and I-RODS Improvement from Stage A Baseline by ECI Response - Stage A Last Assessment
- ARGX-113-1802/1902 Post-hoc Analysis - Table RQ2025023.1A: Mean Grip Strength (dominant hand, kPa): Descriptive Statistics of Actual Values and Changes from Stage A Baseline

The data presented for ADHERE+ are from the interim analysis of 16 February 2024.

Talking Points:

Time to First Improvement

- Among patients who received efgartigimod rHuPH20 SC in ADHERE Stage A, the median time to first improvement was fastest for grip strength (4.1 weeks) compared to aINCAT (8.3 weeks) and I-RODS (6.4 weeks)
 - 50.0% of all patients treated with efgartigimod rHuPH20 SC in Stage A achieved a clinically meaningful improvement in grip strength (≥ 8 kPa in either hand) by 4.1 weeks (95% CI, 3.1-5.1). In comparison, the median time to first improvement for aINCAT (≥ 1 point improvement) was 8.3 weeks (95% CI, 6.6-10.1) and for I-RODS (≥ 4 points improvement) was 6.4 weeks (95% CI, 4.4-8.1).

Confirmed ECI Responders (Stage A) – improvements in mean grip strength

- ECI was defined as an ≥ 8 kPa improvement in either hand for mean grip strength, a ≥ 1 -point improvement for aINCAT, or a ≥ 4 -point improvement for I-RODS. There were 214 patients with ECI in Stage A.
- The majority of patients in ADHERE Stage A with evidence of improvement (ECI) improved in grip strength: 80.4% of patients (172/214) achieved an ≥ 8 kPa improvement in either hand for mean grip strength.
- When combining grip strength with the other functional measures, the percentage of patients who improved across these scales in Stage A is as follows:
 - Mean grip strength and aINCAT: 58.4% (125/214)
 - Mean grip strength and I-RODS: 58.9% (126/214)
 - Mean grip strength, aINCAT, and I-RODS: 46.7% (100/214)

Grip Strength Improvement (Stage A baseline → ADHERE+ Week 36)

- Patients treated with efgartigimod rHuPH20 SC continued to improve in mean grip strength (dominant hand) through Week 36 of ADHERE+, relative to their Stage A baseline values.

Checklist for additional requirements to appropriately describe analyses:

X Type of analysis: Post-hoc

X Analysis set Stage A safety, Intent to Treat

☐ Mean at baseline

☐ Confounders, if applicable

X Limitations of analysis These were post-hoc analyses based on prespecified descriptive supplemental analyses not controlled for multiplicity and not powered and are based on a small sample set; therefore, data should be interpreted with caution and conclusions cannot be drawn.



Approval	
U.S. Medical Leadership	Name: <i>Josie Bryson</i>
	Date: 2026-01-14 22:26 CET
EMA Medical Leadership	Name: <i>Roland Deswand</i>
	Date: 2026-01-14 17:55 CET
Chief Medical Officer	Name: <i>[Signature]</i>
	Date: 2026-01-16 22:18 CET